ANX-01 | Therapist guide only

Mapping before choosing a focus

ANX-01 | A flexible therapist framework, guided by assessment and agreements.

Purpose

Understand worry and its effects on the client's life alongside resources, preferences and strengths. Mapping supports formulation and professional assessment; it is not a diagnostic questionnaire and does not establish that worry is excessive or unfounded.

Before use

Return to the introduction, ASM-A03 and INT-I04 goals as documented. Explore changed circumstances, the current situation and whether this pathway fits the focus and age. Choosing an anxiety card or completing a form does not establish suitability.

A possible working sequence

Open with SES-S01 as needed and close with SES-S02, feedback and a clear plan.

1. Invite description in the client's words and choose one situation or area.
2. Explore topics, contexts, feelings, body sensations and responses without requiring a complete list.
3. Examine effects on participation and functioning alongside living conditions and actual needs.
4. Identify resources and what the client wants to do differently in life.
5. Summarize hypotheses, missing information and a shared next goal; do not diagnose from the sheet.

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Examples for exploration, not ready-made tasks

Examples are fictional. A real client's circumstances, needs and wishes take priority.

Group	A conversation example
Children	A child worries about not understanding a new game's rules and stays out. Explore whether the rules were explained and what the child wants before discussing anxiety.
Adolescents	A teenager worries before submitting work and postpones a hobby. Ask about instructions, workload and personal meaning without assuming anxiety explains every postponement.
Adults	An adult worries about several commitments. Assess practical scheduling problems alongside repeated predictions about the future.
Older adults	A person wants to join an activity but worries about a new arrangement. Explore missing information, access and preferences without drawing conclusions based on age.

A key distinction

Fear of harm, a new health concern or a practical need may require appropriate assessment and response. Body sensations are not automatically anxiety. Do not use the sheet to postpone addressing a real problem.

A question for shared reflection

What does the client want to change, and what do we still need to clarify before choosing an approach?

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Individual adaptation and scope

An age label does not replace assessment of development, communication, context and preferences.

Group	A possible approach
Children, approximately ages 6 to 11	Use a concrete situation and one question at a time. The therapist can write spoken answers and check understanding. Hear the child's voice too. Adult roles, sharing and consent depend on the setting and need; do not promise absolute confidentiality.
Adolescents, approximately ages 12 to 17	Choose a personal focus and recording method that respects privacy. Explore setting, demands and relationships. Explain sharing and consent limits, and arrange help without making practice a test or demand for obedience.
Adults	Include roles, culture, language, circumstances and resources as described. Do not assume time, money or support is available. Shared choice does not replace clinical suitability assessment.
Older adults	Use respectful adult language, comfortable pace and spacing. Ask about preferred completion methods and help without assuming reduced ability or family dependence. Life experience and preferences inform the work.

Checking understanding and choice

Ask the client to explain their understanding and choice in their words. Shorten, leave fields blank or answer aloud. If questions are unsuitable or burdensome, adapt the approach rather than require completion.

This unit's specific boundary

The series does not cover every anxiety disorder. OCD, trauma and other concerns may require separate formulation, training and approaches. Risk assessment or referral needs belong within appropriate professional care, beyond this limited mapping.

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Practice, follow-up and professional review

Practice is chosen jointly; the sheet does not automatically authorize action.

Before practice

Choose one everyday example and brief ANX-01-P recording method with the client. There is no need to seek a difficult event or monitor every worry. A partial memory can be discussed aloud.

Back in the session

Explore what happened, effects on functioning and what helped. If unrecorded, assess demands, understanding and privacy rather than infer low motivation. Next steps may explore responses and consequences in ANX-02 or continue assessment instead.

Production and review status

Original draft guide and sheets. Professional review for each age and a rights review are needed before distribution. Completed writing is not clinical approval; the kit has not been validated or tested for effectiveness.

General sources for further reading

NICE CG113 — adults
Professional framework for adult generalized anxiety: assessment, functioning and work by trained therapists. It does not validate these six units or their use with children.
https://www.nice.org.uk/guidance/cg113/chapter/Recommendations

NIMH — child anxiety, 2024
Research background on CBT for children; the study used a different protocol. It does not approve this wording or these age adaptations.
https://www.nimh.nih.gov/news/science-updates/2024/cognitive-behavioral-therapy-alters-brain-activity-in-children-with-anxiety

Consulted: 03 Nov 2026. Background sources only, without permission to copy forms or validation of the kit.

Reviewer's name, role, date and versions reviewed:

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